NCCP 00331 · VERSION 7

# Your treatment, explained

**Advanced / metastatic colorectal cancer** · NCCP 00331 v7 · Document revised 5 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Fortnightly cetuximab + irinotecan

14-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

## When to get help

### Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

### Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

**Open your CUH contact / treatment card**

Sources: nccp · hse

**Choose a topic to read**

YOUR COMPLETE REFERENCE

# Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Treatment, recovery and review
- Your 14-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Your treatment aim
- Before the first treatment
- Symptoms in context
- Living with treatment
- Visits and treatment sequence
- When treatment changes or ends
- Supportive medicines
- Understanding the evidence
- Questions to ask

Sources

UNDERSTAND YOUR TREATMENT

# How your medicines work

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

EGFR-TARGETED ANTIBODY

## Cetuximab

Erbitux



**Blocks growth messages at the cell surface**

Cetuximab attaches to EGFR on the outside of cells and blocks some growth messages. In this bowel-cancer plan, validated extended RAS testing helps identify whether treatment is suitable. Healthy skin also uses EGFR, so skin and nail effects need early care.

EGFR receives a growth signal → Cetuximab blocks the receptor → Some growth signals are reduced

**What to watch for**

Report rash or sore nails early. Tell the nurse immediately about infusion symptoms. Eye pain, new breathing problems or severe diarrhoea need prompt advice. Blood tests check minerals during treatment and for two months afterwards.

Sources: smpc

TOPOISOMERASE I CHEMOTHERAPY

## Irinotecan

Campto



**Interrupts DNA copying**

The body converts irinotecan into SN-38. This traps an enzyme called topoisomerase I while DNA is being copied, causing damage that can stop cancer cells dividing. Normal bowel and blood-forming cells can also be affected.

Irinotecan becomes SN-38 → DNA-repair enzyme is trapped → Dividing cells are damaged

### What to watch for

Tell the nurse immediately about cramps, sweating or diarrhoea during the infusion. Diarrhoea starting later needs the separate written treatment and contact plan.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

**Explore your existing anatomy and side-effect map**

REGIMEN-SPECIFIC SIDE EFFECTS

# Your anatomical side-effect map

## A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

**1 Eyes**

Cetuximab

Eye pain or new visual change needs prompt advice.

**2 Mouth / swallowing**

Treatment

Pain or ulcers affecting eating or drinking need advice.

**3 Breathing / reaction**

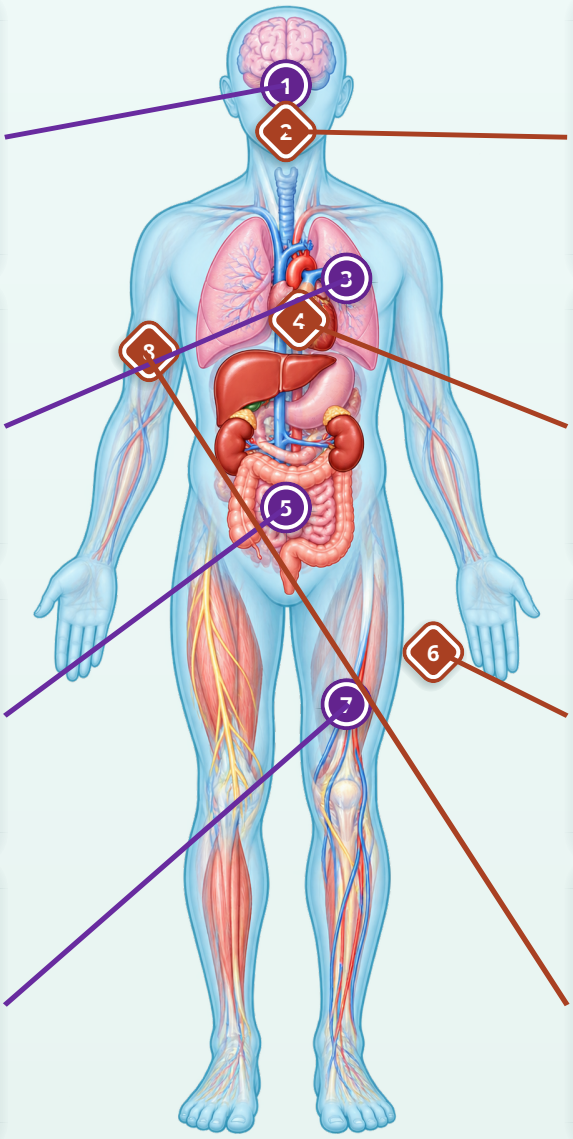
Cetuximab

New breathing difficulty requires prompt assessment.

**4 Chest**

Treatment

New chest pain needs urgent assessment.




**5 Bowel**

Irinotecan

Diarrhoea, increased stoma output or dehydration: call early.

**6 Skin / nails**

Cetuximab

Report painful rash, cracks or infected skin early.

**7 Blood / infection**

Treatment

Fever, shivers or sudden illness: contact oncology immediately.

**8 Infusion access**

Treatment delivery

Pain, swelling, leakage or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

# Treatment, recovery and review

CLINIC → HOME → REVIEW

### A 14-day reference cycle




Cetuximab and irinotecan on Day 1, repeated every 14 days while beneficial and tolerable. There is no Day 8 cetuximab visit and no fluorouracil home pump.

A repeating reference cycle sits within a longer treatment plan. The oncology team chooses the number of cycles and confirms changes; no patient dates are required here.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

| Check | Before starting | Early treatment | Repeated treatment | Longer-term review | Pause / completion |
|---|---|---|---|---|---|
| **Blood counts, kidney and liver** | Baseline blood count, kidney and liver profile | Review results before treatment | Regular blood count, kidney and liver tests as arranged | Repeat sooner if symptoms or results need it | Follow any unresolved abnormal result |
| **Magnesium, calcium and potassium** | Check mineral levels before starting | Repeat periodically during treatment | Keep blood-test and replacement appointments | More checks may be needed if levels fall or diarrhoea develops | Monthly checks for two months after the last antibody dose |
| **Skin, nails, eyes and breathing** | Discuss existing skin, eye or lung problems | Start the prescribed skin-care plan; report reactions early | Review skin, nails, diarrhoea and new symptoms | Eye pain, worsening breathing or severe rash needs prompt assessment | Continue care until symptoms and mineral levels settle |
| **Bowel symptoms and hydration** | Have a written diarrhoea plan and emergency contact | Tell the nurse about cramps, sweating or diarrhoea during infusion | Delayed diarrhoea can start after the first day: act at the first liquid stool using your supplied rescue plan and electrolyte fluids; contact the team early. | Fever, reduced urine, severe pain or inability to drink needs urgent advice | Restart decisions require recovery and team review |
| **Cancer response and treatment choices** | Record baseline disease and agree review plan | Bring forward review if symptoms worsen | Scans and other tests follow your consultant's treatment plan | Discuss benefit, side effects and whether the goal has changed | Agree continuation, a break, another treatment or supportive care |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00331 v7 · official regimen · EMA: Erbitux product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

# Fortnightly cetuximab + irinotecan: treatment and checks

## Advanced / metastatic colorectal cancer

Cetuximab and irinotecan on Day 1, repeated every 14 days while beneficial and tolerable. There is no Day 8 cetuximab visit and no fluorouracil home pump.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Early treatment | Repeated treatment | Longer-term review | Pause / completion |
|---|---|---|---|---|---|
| **Blood counts, kidney and liver** NCCP 00331 | Baseline blood count, kidney and liver profile | Review results before treatment | Regular blood count, kidney and liver tests as arranged | Repeat sooner if symptoms or results need it | Follow any unresolved abnormal result |
| **Magnesium, calcium and potassium** NCCP 00331 | Check mineral levels before starting | Repeat periodically during treatment | Keep blood-test and replacement appointments | More checks may be needed if levels fall or diarrhoea develops | Monthly checks for two months after the last antibody dose |
| **Skin, nails, eyes and breathing** NCCP 00331 | Discuss existing skin, eye or lung problems | Start the prescribed skin-care plan; report reactions early | Review skin, nails, diarrhoea and new symptoms | Eye pain, worsening breathing or severe rash needs prompt assessment | Continue care until symptoms and mineral levels settle |
| **Bowel symptoms and hydration** NCCP 00331 | Have a written diarrhoea plan and emergency contact | Tell the nurse about cramps, sweating or diarrhoea during infusion | Delayed diarrhoea can start after the first day: act at the first liquid stool using your supplied rescue plan and electrolyte fluids; contact the team early. | Fever, reduced urine, severe pain or inability to drink needs urgent advice | Restart decisions require recovery and team review |
| **Cancer response and treatment choices** NCCP 00331 | Record baseline disease and agree review plan | Bring forward review if symptoms worsen | Scans and other tests follow your consultant's treatment plan | Discuss benefit, side effects and whether the goal has changed | Agree continuation, a break, another treatment or supportive care |

## What each check is for

1

### A 14-day reference cycle

Clinic → home → review

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

**Ask / act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00331

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

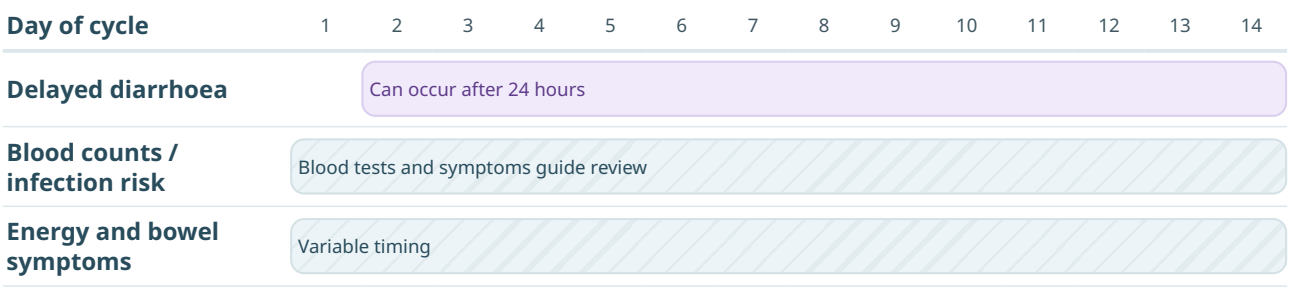
Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.34

## Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Cetuximab and irinotecan on Day 1, repeated every 14 days while beneficial and tolerable. There is no Day 8 cetuximab visit and no fluorouracil home pump.

| Day of cycle | 1 | 2 | 3 | 4 | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 | 14 |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| **Cetuximab infusion** | IV | | | | | | | | | | | | | |
| **Day-ward chemotherapy** | IV | | | | | | | | | | | | | |
| **Skin, nails and minerals** | Can build over successive cycles | | | | | | | | | | | | | |
| **Early cramps / sweating**<br>Tell the nurse | Day 1 | | | | | | | | | | | | | |




**Your planned visits:** Cetuximab on Day 1. Irinotecan is given on Day 1; no home fluorouracil pump. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

**Care continues:** Continue the skin-care plan and mineral checks arranged after cetuximab. Ask which medicines to continue and when your next review is planned.

**No guaranteed safe days:** An infection, reaction or other urgent problem can occur at any time. The graph does not predict a personal low point.

> **Why these bands stay broad:** NCCP defines the treatment days. Symptom bands are broad educational guidance; reactions can occur on any day, and cumulative effects may continue between cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00331 v7 · official regimen ↗️

> **There are no guaranteed "safe days".** Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override any timeline or diary.

**Read the whole cycle as text**

### Day 1: Treatment and checks

The team reviews blood tests, symptoms and your prescription before treatment. Premedicines and observation are part of cetuximab visits. Know the contact number and the next planned visit.

### Day 2–7: Care between visits

Follow the skin-care or bowel-symptom plan provided. There is no pump to remove. Call early for symptoms; do not wait for the next appointment.

### Day 8–14: Recovery and the next review

The team uses symptoms, tests and your priorities to decide what happens next. Bring questions and a symptom record if useful; this is optional.

# Visits, access and care between treatments

Cetuximab and irinotecan on Day 1, repeated every 14 days while beneficial and tolerable. There is no Day 8 cetuximab visit and no fluorouracil home pump.



STEP 1

### Before the first visit

Confirm the exact regimen, tests, medicines, emergency number and treatment goal. Ask whether a peripheral cannula or an existing line is appropriate; a port is not automatically required.



STEP 2

### At the treatment unit

Fortnightly cetuximab infusion time is set from the dose and safe infusion rate; the first dose may take longer. Irinotecan usually takes about 90 minutes. Blood review, preparation, premedicines and observation add time; infusion time is not the total appointment length.



STEP 3

### After the infusion

There is no fluorouracil home pump and no disconnection visit in this regimen.



STEP 4

### Between doses

Use the written symptom and support plan; do not wait for the next appointment if urgent symptoms appear.



STEP 5

### The next decision

Confirm the next visit after any delay or dose change. Treatment continues only while the oncology team judges benefit and tolerability acceptable. Follow-up mineral checks continue after the final EGFR-antibody dose where applicable.

**Transport and daily life**

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

**If treatment changes**

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00331 v7 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

# Support around the medicines

### 1 Skin, nails and minerals

Start the prescribed skin-care plan early, use moisturiser and sun protection, and report painful or infected skin or nails. Keep mineral blood-test appointments during treatment and for two months after cetuximab.

### 2 Bowel symptoms and fluids

Irinotecan can cause early cramps/sweating during infusion and delayed diarrhoea after 24 hours. You need separate instructions for each. Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice. At the first liquid stool, start your individually supplied diarrhoea rescue plan and drink electrolyte-containing fluids as instructed. Contact your team early; do not wait for severe diarrhoea. If you do not have a rescue plan, contact the team immediately for instructions.

### 3 Plan the visit

Allow for blood tests, review, pharmacy preparation, infusion and observation. A port is not automatically required; ask which access is suitable.

### 4 Comfort and daily life

Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

### 5 The whole treatment plan

Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Mineral checks continue monthly for two months after the last antibody treatment.

Sources: nccp · hse

TREATMENT REFERENCE · 01

# Your treatment aim

The setting changes what benefit means.

### Why this option?

This combination is used for RAS wild-type bowel cancer that has spread. This NCCP pathway is a second-line option after oxaliplatin-based treatment has failed or cannot be used. Your team explains the aim, alternatives and how benefit will be reviewed.

Sources: nccp

### A plan that can change

Discuss how treatment aims to control the cancer and symptoms, and whether a response could allow a further local treatment in your individual case. Review benefit, side effects and your priorities as the plan changes.

Sources: nccp

TREATMENT REFERENCE · 02

# Before the first treatment

Checks reduce avoidable risk.

### Blood tests and medicines

Review blood counts, kidney/liver tests, symptoms, allergies and all medicines and supplements. Tell the team about red-meat allergy or tick bites before cetuximab because these can be associated with infusion-reaction risk.

Sources: nccp

### Preparing access

Allow for blood tests, review, pharmacy preparation, infusion and observation. A port is not automatically required; ask which access is suitable.

Sources: nccp

TREATMENT REFERENCE · 03

# Symptoms in context

Report what changed and how it affects you.

### When to call

New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly.

Sources: nccp · hse

### Bowel symptoms

Irinotecan can cause early cramps/sweating during infusion and delayed diarrhoea after 24 hours. You need separate instructions for each. Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice. At the first liquid stool, start your individually supplied diarrhoea rescue plan and drink electrolyte-containing fluids as instructed. Contact your team early; do not wait for severe diarrhoea. If you do not have a rescue plan, contact the team immediately for instructions.

Sources: nccp

### Skin and nail changes

Start the prescribed skin-care plan early, use moisturiser and sun protection, and report painful or infected skin or nails. Keep mineral blood-test appointments during treatment and for two months after cetuximab.

Sources: nccp

TREATMENT REFERENCE · 04

## Living with treatment

Use the support available.

### Food, activity and practical help

Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

Sources: nccp · hse

**Pregnancy and fertility**

Discuss fertility preservation and reproductive plans before treatment. Ask for the contraception and breastfeeding instructions for every medicine; the whole regimen may require longer precautions than one medicine alone.

Sources: nccp

TREATMENT REFERENCE · 05

## Visits and treatment sequence

Know which steps happen in the clinic and at home.

**The exact delivery plan**

Cetuximab on Day 1. Irinotecan is given on Day 1; no home fluorouracil pump. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Sources: nccp

**Radiotherapy relationship**

This NCCP regimen does not contain a routine radiotherapy course. If radiation is recommended separately, ask whether it is before, during or after these medicines.

Sources: nccp

TREATMENT REFERENCE · 06

## When treatment changes or ends

Follow-up continues.

**Symptoms and monitoring**

Continue the cetuximab skin plan and mineral checks for two months after the last dose. Ask which medicines to continue, when to seek help and how response will be assessed.

Sources: nccp

**Next decisions**

The team reviews benefit, side effects and your priorities. Supportive and palliative care can help alongside active treatment. Ask for a clear next-step and contact plan.

Sources: nccp · hse

# EPIC: extended RAS analysis

Studies can help explain why a treatment is offered. The schedule, previous medicines and tumour tests affect how well the evidence fits you.

Adding cetuximab to irinotecan improved tumour shrinkage and the time before cancer worsened in a selected trial group. Overall survival was similar between the groups. Many patients in the comparison group later received cetuximab. The trial used a different irinotecan schedule from this NCCP plan.

Sources: trial

PREPARE FOR THE CONVERSATION

# Questions worth asking

**How is my treatment chosen?**

This combination is used for RAS wild-type bowel cancer that has spread. This NCCP pathway is a second-line option after oxaliplatin-based treatment has failed or cannot be used. Your team explains the aim, alternatives and how benefit will be reviewed.

**Do I need to enter dates to understand the cycle?**

No. The graph explains a general pattern. Your hospital arranges appointments; the guide and its safety information are available without entering anything.

**Will I have a pump at home?**

There is no home fluorouracil pump in this regimen. Your team chooses a cannula or central line based on your veins and wider treatment plan.

**What happens if a dose is delayed?**

The team checks the cause and may delay, reduce, change or stop one medicine. Ask which component changes and when you should next attend; do not restart yourself.

**How will I know whether it is helping?**

Scans, symptoms and clinical review assess benefit. A rash, side effect or good day alone cannot show whether the cancer is responding.

**What about radiotherapy?**

This NCCP regimen does not contain a routine radiotherapy course. If radiation is recommended separately, ask whether it is before, during or after these medicines.

**What about fertility, pregnancy and breastfeeding?**

Discuss fertility preservation and reproductive plans before treatment. Ask for the contraception and breastfeeding instructions for every medicine; the whole regimen may require longer precautions than one medicine alone.

**Why is RAS testing mentioned?**

RAS testing selects cetuximab treatment in colorectal cancer. It is not the selection test for the separate head-and-neck radiotherapy indication. Confirm the cancer-specific plan with your team.

> **Try explaining the plan back:** Can you explain which visits are planned, whether a pump is involved, and which symptoms mean you should call today? No dates or symptom scores are needed to use this guide.

SOURCE TRANSPARENCY

# Where this information comes from

Regimen schedule: NCCP 00331 v7. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00331 v7 · official regimen
- HSE: chemotherapy side effects and urgent advice
- EMA: Erbitux product information and patient leaflet
- EPIC extended RAS primary report
- NCCP NCIS supportive antiemetic medicines V8 (2025)

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

## Written source addresses

https://healthservice.hse.ie/documents/6786/331_v7_Cetuximab_14days_irinotecan_14_day.pdf

https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://academic.oup.com/oncolo/article/26/2/e261/6445358

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf